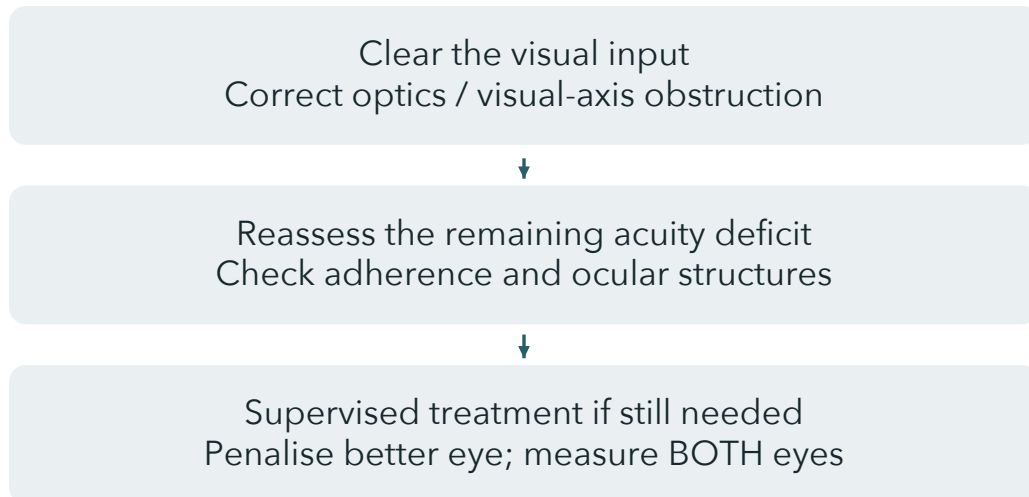


Amblyopia: response and treatment safety

CLINICAL TYPE B | Focused decisions for M3-M5

Treat unequal input, then measure the response

A treatment loop, not a one-off patch



New fellow-eye loss -> review and interrupt therapy

Original clinical sequence. Penalisation targets the better-seeing eye under supervision.

Before increasing treatment

Finding	Clinical inference
Optical correction	Confirm prescription, wearing and an adequate adaptation period.
Persistent deficit	Recheck structures, acuity reliability and adherence; specialist patching or atropine may be considered.
An older untreated child	A supervised trial can still help some adolescents; do not impose a rigid age-seven cutoff.

References AAO: Amblyopia Preferred Practice Pattern | PEDIG: Amblyopia treatment at ages 7-17

Monitor the eye you are penalising

When fellow-eye acuity falls

- 1** Retest acuity and refraction in BOTH eyes; assess for reverse amblyopia or another cause.
- 2** Interrupt active penalisation and review promptly. If loss persists, investigate optic-nerve or macular disease.
- 3** Monitor recovery before restarting a revised regimen; do not automatically increase or reverse the patching dose.

Follow-through protects the gains

Finding	Clinical inference
Improvement	Continue appropriate optical correction and monitor both eyes.
Plateau	Review adherence and diagnosis before changing intensity.
Tapering / stopping	Plan follow-up for recurrence rather than discharging on one good acuity measurement.

What alignment surgery does

Surgery may improve alignment or clear a blocked visual axis, but it does not replace visual rehabilitation for an established developmental acuity deficit.